



P50 · 40 PAGES, PRINTABLE + FILLABLE

The Family Caregiver Workbook

A forty-page workbook for the long-haul family caregiver.

The ongoing months and years of caring for an aging parent or a partner with chronic illness. Updated medical team contacts, monthly care-team coordinator log, respite-care planning, the honest pages for anger and exhaustion, the sibling-conflict workbook, marriage and career considerations, and a "what they would want, if they could tell me" page. Companion to the 90-Days planner. Caregiver crisis lines included.

Soothemade Notes

A SMALL APOTHECARY OF PRINTABLES · MADE SLOWLY

A PLANNING AND TRACKING TOOL · NOT MEDICAL ADVICE

A note before you start

The first 90 days were the shock. This is the long stretch.

You have settled into a rhythm. The appointments are routine. The medications are mostly stable. You and the medical team know each other. The family has its pattern.

What you may not have is sustainability. The person you are caring for has needs that do not go away. The caregiver work is now your second job, on top of your first. Your marriage may be strained. Your own health may be slipping. Your siblings may be doing too little or too much, in ways you have not had the energy to fight about.

This workbook is for the version of you doing this for the long haul. The structure to make it sustainable. The honest pages for the anger and the exhaustion. The reckoning page for the day you realize you cannot do this forever and the question of what comes next.

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This is a planning tool, not medical or legal advice. The clinical decisions in your loved one's care belong to them, the medical team, and you and your family acting on their behalf. Financial and legal decisions belong to you and the professionals working with you. This workbook does not name medications. The crisis-line page is at the back. Caregivers are at meaningfully elevated risk for depression, anxiety, substance use, and their own health collapse. Knowing this in advance is one of the only ways to prevent it.

How to use this workbook

- Print the whole workbook, or re-print pages monthly as the situation changes.
- The medication log and the monthly coordinator log are designed to be re-printed.
- The honest pages (anger, exhaustion, cannot-do-this-forever) are for you alone. They are not for the loved one to see. They are not for the siblings to see, except by your choice. Burn them or shred them if you want. They are for you to know what is true.
- The respite planning page is the most important page in this workbook. Sustainability requires actual time off.

You are doing real work

A page to come back to.

A REMINDER

This is a job.

It is unpaid. It is largely unseen. It does not have a title that adequately describes it. It does not have a contract or sick days or a clear end date. It is not what you signed up for. It is what you showed up for.

The work includes:

- Medical advocacy (knowing the team, the medications, the conditions, the warning signs)
- Logistics (appointments, transportation, paperwork, insurance)
- Direct care (the physical tasks, whatever they are)
- Emotional labor (being present, holding their fear, holding yours)
- Social work (managing visitors, family, friends)
- Financial management (their finances and yours)
- Project management (the household systems your caregiving requires)

These are skills. They count, even when no one outside this house sees them. They are also tiring, even when you would not trade the role.

WRITE DOWN, RIGHT NOW, THREE THINGS YOU HAVE LEARNED HOW TO DO THAT YOU DID NOT KNOW HOW TO DO A YEAR AGO

1. _____
2. _____
3. _____

THESE ARE REAL CHANGES IN YOU. THEY WILL OUTLAST THIS CHAPTER.

Medical team contacts, updated

Re-fill this page every six months. The team changes.

PRIMARY CARE

Name: _____

Phone: _____

SPECIALIST (main): _____

Name: _____

Phone: _____

SPECIALIST 2:

Name: _____

Phone: _____

SPECIALIST 3:

Name: _____

Phone: _____

HOME HEALTH AGENCY (if applicable)

Agency name: _____

Primary nurse: _____

Phone: _____

PHYSICAL / OCCUPATIONAL / SPEECH THERAPY

PT: _____

OT: _____

Speech: _____

PHARMACY

Pharmacy: _____

Pharmacist who knows the case: _____

Phone: _____

HOSPICE (if relevant or to consider later)

Hospice agency: _____

Phone: _____

SOCIAL WORKER

Name: _____

Phone: _____

ANY NEW MEMBER OF THE TEAM THIS MONTH

Medication log by purpose

The medications change. Re-print this page when they do.

Monthly care-team coordinator log

Who is doing what, this month. Re-print monthly.

MONTH OF: _____

WHO IS DOING WHAT

PRIMARY CAREGIVER (most days): _____

BACKUP CAREGIVER (when primary is off): _____

PAID HELP (if any):

Name: _____

Hours per week: _____

What they do: _____

SIBLING / FAMILY MEMBER 1: _____

What they do: _____

Days / times: _____

SIBLING / FAMILY MEMBER 2: _____

What they do: _____

Days / times: _____

VOLUNTEER OR FRIEND HELPERS:

GAPS IN THE COVERAGE (what is not getting covered)

WHAT I AM ASKING FOR HELP WITH THIS MONTH (be specific, use P49 scripts if needed)

Insurance + billing tracker

Re-print monthly or quarterly.

THIS MONTH'S BILLS

DATE	FROM	AMOUNT	STATUS
_____	_____	\$____	paid/pending
_____	_____	\$____	paid/pending
_____	_____	\$____	paid/pending
_____	_____	\$____	paid/pending
_____	_____	\$____	paid/pending
_____	_____	\$____	paid/pending
_____	_____	\$____	paid/pending
_____	_____	\$____	paid/pending

INSURANCE QUESTIONS THIS MONTH

Claim disputed: _____

Status: _____

Pre-authorization needed: _____

Out-of-network surprise: _____

Pharmacy benefit issue: _____

WHO TO CALL

Insurance member services: _____

Insurance billing department: _____

Patient advocate (if available): _____

A NOTE: PATIENT ADVOCATES EXIST IN MOST LARGER HOSPITALS AND HEALTH SYSTEMS, AND THEY CAN BE SHOCKINGLY EFFECTIVE AT RESOLVING BILLING ERRORS, INSURANCE DENIALS, AND CARE COORDINATION ISSUES. ASK FOR ONE BY NAME.

Respite-care planning

The most important page in this workbook. Sustainability requires actual time off.

RESPIRE OPTIONS

The kinds of respite available to caregivers:

- ☐ A family member taking over for a stretch (a sibling for a weekend, an adult child for a vacation)
- ☐ Paid in-home help for a day or weekend
- ☐ Adult day care (the loved one goes to a facility for the day, comes home in the evening)
- ☐ A short-stay at a facility (some assisted living and skilled nursing facilities offer "respite stays" of a few days to weeks)
- ☐ Hospice respite (a Medicare benefit in the US for those in hospice care, allows the patient to stay at a facility for up to 5 days)
- ☐ Specialized respite programs (some non-profits and disease-specific organizations offer respite for caregivers of people with certain conditions, e.g. Alzheimer's)

WHAT I AM RESERVING, RIGHT NOW

The next time I will get a real day off: _____

Who is covering: _____

The next time I will get a real weekend off: _____

Who is covering: _____

The next time I will be gone overnight: _____

Who is covering: _____

The next vacation (real, more than 3 days): _____

Who is covering: _____

LOCAL RESPITE RESOURCES

Area Agency on Aging (US): _____

(Find via 1-800-677-1116)

Disease-specific respite (e.g. Alzheimer's Association):

Adult day care option: _____

Phone: _____

Cost per day: _____

Short-stay facility option: _____

Phone: _____

WHAT IS BLOCKING ME FROM USING RESPITE

- ☐ Cost
- ☐ Guilt
- ☐ The loved one resists
- ☐ I cannot find the right option
- ☐ I cannot find someone to cover
- ☐ I have not asked, because I am scared the answer is no

For each box you checked, the workbook has a page that helps you work on it. Start there.

A NOTE: CAREGIVERS WHO USE REGULAR RESPITE STAY IN THE CAREGIVING ROLE LONGER. CAREGIVERS WHO DO NOT USE RESPITE OFTEN COLLAPSE AND THE LOVED ONE ENDS UP IN A HIGHER LEVEL OF CARE THAN WAS OTHERWISE NECESSARY. RESPITE IS NOT SELFISH. IT IS THE TOOL THAT KEEPS THIS SUSTAINABLE.

The “I am angry” page

This page is for you alone. Burn it or shred it if you want.

WHAT I AM ANGRY ABOUT, RIGHT NOW

THE PEOPLE I AM ANGRY AT, AND WHY (no diplomacy)

THE THINGS I AM SECRETLY ANGRY AT MY LOVED ONE ABOUT

This is the page nobody writes. The page that says, "I am angry at the person I am caring for, even though they did not choose this either." That anger is real. It does not make you a bad caregiver.

THE THINGS I AM ANGRY AT MYSELF ABOUT

ONE WAY I COULD MOVE SOME OF THIS ANGER

- ☐ Talking to a therapist
- ☐ Talking to a caregiver support group
- ☐ A long walk by myself
- ☐ Writing more pages of this
- ☐ A real conversation with a sibling I am angry at
- ☐ A real conversation with my partner

- ☐ Hitting a pillow (yes, really)
- ☐ Something else: _____

A REMINDER: ANGER IS NOT CHARACTER. IT IS INFORMATION ABOUT WHAT IS TOO MUCH. THE INFORMATION IS USEFUL EVEN WHEN THE ANGER ITSELF IS NOT FUN TO HOLD.

The “I am exhausted” page

WHAT EXHAUSTION FEELS LIKE, RIGHT NOW

THE LAST TIME I FELT RESTED, REALLY

HOW MANY MONTHS AGO WAS THAT

____ months

WHAT I AM RUNNING ON

- ☐ Coffee
- ☐ Adrenaline
- ☐ Sheer stubbornness
- ☐ The fear of what happens if I stop
- ☐ Real rest

(You probably can't check the last one.)

WHAT I HAVE GIVEN UP THAT USED TO RESTORE ME

WHICH ONE COULD I GET BACK, FIRST, IF I MADE IT A PRIORITY

WHAT IT WOULD TAKE

A REMINDER: EXHAUSTION IS NOT A PERSONALITY TRAIT. IT IS A SIGN THAT THE WORK IS BIGGER THAN THE REST. THE FIX IS REST, NOT TRYING HARDER.

The “I cannot do this forever” page

The hardest page in the workbook.

THE THOUGHT I AM NOT TELLING ANYONE

WHAT THE THOUGHT IS ACTUALLY ABOUT

- ☐ I cannot keep up this level of care
- ☐ I want my life back
- ☐ I am scared they will outlive my own health
- ☐ I am scared my marriage will not survive this
- ☐ I am scared I will not survive this
- ☐ I love them and I am also done
- ☐ Other:

WHAT WOULD HAVE TO CHANGE FOR ME TO BE OKAY

- ☐ More respite
- ☐ More paid help in the home
- ☐ A move to a different level of care for the loved one
- ☐ A different sibling arrangement
- ☐ My own therapist or counselor
- ☐ A real time-off plan
- ☐ Acceptance of help I have been refusing
- ☐ A different kind of conversation with the team about prognosis
- ☐ All of the above

THE CONVERSATION I NEED TO HAVE WITH

- | | |
|------------------------------|--|
| My partner: | ☐ scheduled ☐ avoiding |
| My sibling(s): | ☐ scheduled ☐ avoiding |
| The medical team: | ☐ scheduled ☐ avoiding |
| A therapist: | ☐ scheduled ☐ avoiding |
| The loved one (if able): | ☐ scheduled ☐ avoiding |
| Myself (the honest version): | ☐ done ☐ avoiding |

THE NEXT 30 DAYS

What I am willing to put in motion:

A NOTE: ADULT CHILDREN WHO MOVE A PARENT TO A HIGHER LEVEL OF CARE ARE NOT FAILING. THEY ARE OFTEN MAKING THE DECISION THE PARENT WOULD HAVE MADE FOR THEMSELVES, IF THE PARENT COULD STILL DECIDE.

PARTNERS WHO RECRUIT MORE PAID HELP ARE NOT GIVING UP. THEY ARE RECOGNIZING THAT ONE PERSON IS NOT ENOUGH FOR THIS WORKLOAD.

YOU ARE ALLOWED TO BE HONEST ABOUT THE LIMIT, EVEN WHILE STILL LOVING THEM.

Sibling-conflict workbook, page 1

For families with more than one adult child involved. Often the source of more exhaustion than the caregiving itself.

THE WORKLOAD DIVISION, IN PLAIN WORDS

What I am doing per week (in hours): _____

What sibling 1 is doing per week: _____

What sibling 2 is doing per week: _____

What sibling 3 is doing per week: _____

What paid help is doing per week: _____

Is this balanced? ☐ Yes ☐ No

If not, what is the imbalance:

THE OLD STORY OUR FAMILY TELLS ABOUT EACH OF US

About me: _____

About sibling 1: _____

About sibling 2: _____

About sibling 3: _____

Which old story is shaping the current workload?

WHAT EACH PERSON BRINGS THAT THE OTHERS DO NOT

Me: _____

Sibling 1: _____

Sibling 2: _____

Sibling 3: _____

WHAT IS REASONABLE TO ASK OF EACH PERSON

This is not about fairness in hours. It is about contribution that matches capacity. A sibling 1,000 miles away cannot do the same hours as the local sibling. They can do other things.

Sibling-conflict workbook, page 2

THE CONVERSATIONS I NEED TO HAVE

WITH SIBLING 1

The thing I want them to know:

The thing I want them to do differently:

The thing I am willing to do differently:

WITH SIBLING 2

The thing I want them to know:

The thing I want them to do differently:

The thing I am willing to do differently:

WITH SIBLING 3

The thing I want them to know:

The thing I want them to do differently:

Sibling-conflict workbook, page 3

THE FAMILY MEETING

When: _____
Where: _____
Who: _____

The agenda I am bringing:

1. Update on the loved one
2. The current workload split
3. The needs we are not meeting
4. The proposal I have for redistribution
5. The questions for each sibling
6. The next 60-day commitments

WHAT I AM PROPOSING

THE BACKUP PLAN IF THIS MEETING DOES NOT WORK

- ☐ A family therapist or elder-care mediator
- ☐ A written agreement, even if informal
- ☐ Hiring more paid help
- ☐ Stepping back myself in a meaningful way
- ☐ Other: _____

Sibling-conflict workbook, page 4

THE THINGS I AM NOT GOING TO REPAIR RIGHT NOW

Sometimes the family wound is older than the current caregiving situation. Sometimes the wound is being created by the caregiving situation. Sometimes both.

The wound I am setting down for now, even if I will pick it up later:

What I am willing to forgive, or set aside, for the duration:

What I am NOT willing to forgive, even when this is over:

A NOTE: SOME RELATIONSHIPS DO NOT SURVIVE A FAMILY CAREGIVING EPISODE. SOME GET STRONGER. WHICH IT WILL BE IS NOT ENTIRELY UP TO YOU. WHAT IS UP TO YOU: HOW YOU SHOW UP FOR THE CONVERSATIONS, HOW HONEST YOU ARE, HOW MUCH YOU LET THE OLD FAMILY STORY DECIDE WHO YOU ARE NOW.

*What they would want, if they could tell
me*

For caregivers of loved ones with dementia or advanced illness. The decisions you make on their behalf are real decisions, weighted by what they would want.

WHAT THEY USED TO SAY ABOUT GETTING OLD

WHAT THEY USED TO SAY ABOUT MEDICAL INTERVENTION

WHAT THEY USED TO SAY ABOUT NURSING HOMES OR FACILITIES

WHAT THEY USED TO SAY ABOUT FUNERALS

WHAT VALUES THEY LIVED BY

THE DECISION I HAVE TO MAKE THIS MONTH

WHAT THEY WOULD CHOOSE, IN MY BEST GUESS

A NOTE: YOU ARE NOT BETRAYING THEM BY MAKING A HARD CHOICE. YOU ARE EXTENDING WHO THEY WERE INTO THE DECISIONS THEY CAN NO LONGER MAKE. THIS IS THE SACRED PART OF CAREGIVING.

Marriage / partnership maintenance

Caregiving is a leading cause of marital strain and divorce. The work has to be done to keep the marriage standing.

HONEST CHECK ON OUR MARRIAGE / PARTNERSHIP

How often we have a real conversation per week: ____

How often we have one that is NOT about caregiving: ____

How often we are physically affectionate: ____

How often we go out together (alone, no kids/no
caregiving): ____

The thing I miss most about us before this:

The thing my partner has been carrying that I have
not acknowledged:

The thing I have been carrying that my partner has not
acknowledged:

WHAT WE COULD COMMIT TO, THIS MONTH

- ☐ One date night, real, away from the house and the loved one
- ☐ One conversation per week that is just about us
- ☐ Couples counseling (worth investing in, often)
- ☐ A weekend away (real respite, planned)
- ☐ Reading the same book together
- ☐ Going to bed at the same time

THE COUPLES THERAPIST I AM RESERVING THE NUMBER FOR

A NOTE: COUPLES COUNSELING IS THE SINGLE BEST INVESTMENT MANY
CAREGIVING COUPLES MAKE. IT IS NOT A FAILURE. IT IS A MAINTENANCE
TOOL FOR THE PARTNERSHIP MOST LIKELY TO ABSORB THIS LOAD.

Job and career considerations

For caregivers still in paid work.

THE CURRENT JOB SITUATION

Hours per week, in paid work: _____

Flexibility I have: _____

Manager who knows the caregiving situation: _____

Family Medical Leave (US) used so far: _____ weeks

WHAT I HAVE GIVEN UP AT WORK

WHAT I AM NOT GIVING UP, NO MATTER WHAT

OPTIONS TO CONSIDER

- ☐ Family Medical Leave (intermittent or block, US)
- ☐ Reduced schedule
- ☐ Switching to a more flexible role
- ☐ A different employer with better caregiving support
- ☐ A career pause (with eyes open to financial and re-entry implications)
- ☐ Leveraging employer's Employee Assistance Program (EAP)

THE CONVERSATION WITH MY MANAGER OR HR

When: _____

What I plan to say:

What I am asking for:

A REMINDER: HR PROFESSIONALS WHO HANDLE FMLA REQUESTS HAVE SEEN EVERY CAREGIVING SITUATION. THEY ARE OFTEN HELPFUL ALLIES, NOT ADVERSARIES. ASK SPECIFICALLY: "WHAT OPTIONS DO I HAVE FOR CAREGIVING LEAVE?"

Your own health

Caregivers neglect their own medical care. This is well-documented and a leading cause of caregiver health collapse.

WHEN WAS MY LAST

Primary care visit: _____
Dental cleaning: _____
Eye exam: _____
Mammogram / prostate exam (if relevant age): _____
Colonoscopy (if relevant age): _____
Therapy session: _____
Real exercise: _____
Real meal sitting down: _____
Real sleep night: _____

ANY APPOINTMENTS I HAVE BEEN DELAYING

SYMPTOMS I HAVE BEEN IGNORING

THE NEXT THREE APPOINTMENTS I AM SCHEDULING

1. _____
2. _____
3. _____

A REMINDER: THE LOVED ONE NEEDS YOU TO BE WELL. YOU NEED YOU TO BE WELL. THE APPOINTMENTS YOU ARE DELAYING ARE NOT OPTIONAL. THEY ARE PART OF KEEPING THIS SUSTAINABLE.

Financial caregiving + hospice considerations

The ongoing financial picture, and the eventual end-of-life considerations.

ONGOING CAREGIVING COSTS, MONTHLY

Paid help: \$____
Facility (if applicable): \$____
Medications: \$____
Medical equipment / supplies: \$____
Transportation: \$____
Specialized food / supplements: \$____
Insurance copays: \$____
Out-of-network or uncovered expenses: \$____

MONTHLY TOTAL: \$____

ANNUAL TOTAL: \$____

LONG-TERM-CARE INSURANCE STATUS

In place: ☐ yes ☐ no ☐ unclear
Benefits available: \$____
What it covers: _____

MEDICAID / MEDICARE STATUS

Medicare: ☐ yes
Medicaid (US): ☐ yes ☐ no ☐ in application
Other government program: _____

HOSPICE CONSIDERATIONS (for the day when this becomes relevant)

Many people qualify for hospice earlier than they realize. Hospice in the US is largely covered by Medicare. It is NOT just "the last week of life," it can be months of comfort-focused care, often at home, often dramatically improving the loved one's quality of life and the caregiver's burden.

What I want to ask the medical team about hospice eligibility:

When I want to start the conversation: _____

A NOTE: THE STIGMA AROUND HOSPICE ("GIVING UP") IS LARGELY UNFOUNDED. HOSPICE IS A COMFORT-FOCUSED CARE TEAM. PEOPLE SOMETIMES STABILIZE OR EVEN IMPROVE ONCE THE FOCUS SHIFTS FROM CURATIVE TO COMFORT. THE CAREGIVER BURDEN OFTEN DECREASES MEANINGFULLY UNDER HOSPICE. WORTH HAVING THE CONVERSATION EARLY.

Caregiver crisis lines

CALL THESE IF YOU NEED THEM

US, 988 Suicide and Crisis Lifeline (call or text)

US, Eldercare Locator (free): 1-800-677-1116

Connects you to your local Area Agency on Aging, which has caregiver support programs, respite resources, and more.

US, Family Caregiver Alliance: 1-800-445-8106

Provides caregiver-specific support and resources.

US, Alzheimer's Association 24/7 Helpline: 1-800-272-3900

For dementia-related caregiving specifically. they also help with non-Alzheimer's dementias.

US, SAMHSA National Helpline (substance use): 1-800-662-4357

Caregivers have elevated rates of alcohol use. This line is free, confidential, 24/7.

UK, Carers UK: 0808 808 7777

UK, Samaritans: 116 123

Canada, Carers Canada / Canadian Hospice Palliative Care Association (find local programs)

Canada, 9-8-8 Suicide Crisis Helpline

Australia, Carer Gateway: 1800 422 737

YOUR OWN THERAPIST (reserve a number now)

Name: _____

Phone: _____

A specialty in caregiver burnout, family-systems, or grief is useful.

A REMINDER: CAREGIVERS OFTEN DELAY SEEKING HELP UNTIL THEY ARE IN CRISIS. THE EARLIER YOU INVOLVE A THERAPIST, A SUPPORT GROUP, OR A CRISIS LINE, THE BETTER THE OUTCOMES. THIS IS NOT WEAKNESS. THIS IS

PROFESSIONAL ATHLETES HAVING TRAINERS. THE WORK IS HARD. THE SUPPORTS ARE THE TOOLS THAT MAKE IT POSSIBLE.

A final letter

You are doing one of the hardest things a human being does.

You are sustaining the life and dignity of someone who can no longer fully sustain themselves. You are doing it in a culture that does not always acknowledge the work. You are doing it on top of your own life, your own marriage if you have one, your own career if you have one, your own children if you have them. You are doing it for months, sometimes years, often without anyone saying "I see what you are doing and I am grateful."

I see what you are doing. I am grateful.

The workbook does not make this easier. It is a tool, not a cure. But it is also a record. A record of the time you took to think about this honestly. A record of the conversations you had the courage to have, with siblings, with partners, with yourself. A record of the moments you knew you were exhausted, knew you were angry, knew you could not do this forever, and still came back the next day.

When this chapter closes, whichever way it closes, you will look back on what you did. The workbook will still be here. You will read what you wrote in your hardest moments and you will be amazed at the person who showed up.

You are still that person. You are still showing up.

The workbook closes here. The work continues. So do you.

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Soothemade *Notes*

*Made for the version of you no one tells you about, the
one at four in the morning, the one in the parking lot, the
one who doesn't recognize her own week.*

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